

Touch in Every Body

Body size, pressure tolerance, and the equipment and awareness the spa industry left out for too long

Learning Objectives

- Adapt equipment, positioning, and draping confidently for a full range of body sizes
- Calibrate pressure based on individual tolerance rather than a single default setting
- Recognise and respond appropriately to touch aversion or trauma-informed needs
- Explain why "one table setup fits everyone" was never actually true

A body of knowledge built for one body type

For much of the spa industry's history, equipment, training, and even table weight ratings were designed around a narrow assumption of what a "typical" client's body looked like — leaving larger-bodied clients underserved, uncomfortably positioned, or quietly turned away. This module exists to correct that inheritance directly, the same way Course I corrected skincare's blind spot on deeper skin tones.

Equipment and positioning for every body size

- **Table weight capacity.** Know your table's rated weight limit precisely, and invest in a bariatric-rated table if your practice serves a general population — never guess, and never make a client feel like an afterthought by hesitating visibly at this question.
- **Bolster and positioning adjustments.** Larger bodies, pregnant bodies, and bodies with joint limitations may all need additional or repositioned bolsters for comfortable, supported prone or side-lying positioning — ask rather than assume a single default setup works.

- **Draping adjustments.** Larger-bodied and smaller-bodied clients alike deserve draping that remains fully secure through repositioning — practice draping technique on a range of body sizes, not just one.

Pressure tolerance is individual, not standard

"Medium pressure" means something different for a client who exercises intensely and carries dense muscle mass than for a client on blood-thinning medication or with thin, fragile skin.

Ask directly at the start of every service, check in during it, and adjust without ego — a client who quietly endures pressure that is too firm, or too light to feel effective, rarely says so unprompted, and will simply not return.

◆ Figure 2.1 · Pressure Is a Spectrum, Not a Default



Ask, check in, and adjust — never assume where on this spectrum a client sits.

Touch aversion and trauma-informed awareness

Not every client wants extended, close physical contact, regardless of how the service is typically performed, and some clients carry a history — medical, personal, or cultural — that makes touch, particularly in certain areas or positions, genuinely difficult. Signs include visible tension that does not ease, guarded breathing, or a client repeatedly steering conversation away from the treatment itself. Respond by explicitly offering control: "Tell me any time to adjust pressure, skip an area, or stop entirely — this is entirely on your terms today." Never interpret silence as consent to continue exactly as planned; check in genuinely, more than once, across a longer service.

Key Terms

[object Object]

1

[object Object]

2

[object Object]

Practical Activity · The Inclusive Setup

Practice adjusting your table, bolster placement, and draping for at least three different body sizes or positioning needs using a partner or props. Write down one specific check-in question you would ask before beginning any service, and practice saying it aloud naturally.